



Australian Obesity & GLP-1 Market Context

Demographic and clinical context for the Melbourne and Victorian patient population.
Prepared for internal reference — Meridian Health Services.

01 — NATIONAL SNAPSHOT

Australian Obesity Landscape

32%

Australian adults classified as obese (BMI 30+)

~13%

Severely obese adults — BMI 35 or above

34%

Additional adults classified as overweight

~7M

Australians living with obesity

~3M

Estimated adults meeting BMI 35+ clinical threshold

~500k

Australians using GLP-1 medications monthly (2026 est.)

Australia ranks among the fastest-growing GLP-1 markets globally. Health Minister Mark Butler confirmed approximately 400,000 Australians are paying full private market price for GLP-1 medications, with UNSW-linked research estimating 180,000–240,000 privately accessing them monthly. An estimated 6 million units were dispensed in the 2024–25 year.

02 — VICTORIAN & MELBOURNE MARKET SIZING

Regional Population & Clinical Cohort

VICTORIA — EST. 7 MILLION

Overweight or obese adults **~2.2M**

Obese adults (BMI 30+) **~900k**

Severe obesity cohort (BMI 35+) **~350k**

GREATER MELBOURNE — EST. 5.5 MILLION

Overweight or obese adults **~1.8M**

Obese adults (BMI 30+) **~750k**

Severe obesity cohort (BMI 35+) **~280k**

These cohorts represent the primary clinical population for bariatric surgery, metabolic medicine, GLP-1 programs, and multidisciplinary obesity care. The Melbourne-area BMI 35+ cohort alone — estimated at 220,000–280,000 adults — represents a substantially underpenetrated surgical and metabolic treatment market.



03 — GLP-1 PRESCRIBING CRITERIA IN AUSTRALIA (2026)

Who Qualifies for GLP-1 Therapy

PRIMARY CRITERION

BMI ≥ 30

Obesity diagnosis, no comorbidity required

This criterion alone places an estimated 6–7 million Australians within medical obesity treatment eligibility.

SECONDARY CRITERION (WITH COMORBIDITY)

BMI ≥ 27

Overweight, with weight-related condition

- Type 2 diabetes
- Obstructive sleep apnoea
- Hypertension
- Hypercholesterolaemia
- Polycystic ovarian syndrome (PCOS)
- Cardiovascular disease

Combined, these criteria potentially place 7–9 million Australians within the scope of medical obesity treatment. GLP-1 awareness has broadly expanded patient engagement with weight management pathways, increasing the number of adults seeking clinical assessment.

04 — CLINICAL RATIONALE

Why Bariatric Surgery Remains Clinically Relevant

- 1 GLP-1 medications are expanding the referral funnel**
GLP-1 uptake has increased awareness of weight-related conditions, reduced stigma, and brought patients into treatment earlier. Many of those patients plateau, discontinue due to cost, or remain severely obese — and in time require stronger metabolic intervention. This creates a sustained pipeline for sleeve gastrectomy, gastric bypass, revisional surgery, and multidisciplinary metabolic programs.
- 2 Australia's severe obesity cohort remains largely untreated**
With approximately 13% of Australian adults at BMI 35 or above — roughly 2.7–3 million people — the addressable surgical and metabolic treatment market remains substantially underpenetrated. Only a small fraction undergoes bariatric surgery annually, representing a significant unmet clinical need.
- 3 GLP-1 discontinuation rates are high**
2026 data indicates approximately 50% of patients cease GLP-1 therapy within a year, with rapid weight regain commonly following cessation. This pattern is driving a clinical shift toward combination care models — long-term metabolic pathways that pair surgical intervention with GLP-1 maintenance protocols.
- 4 The field is shifting toward combination and continuity care**
The emerging clinical consensus integrates GLP-1 medications within broader metabolic care models rather than positioning them as standalone treatments. Bariatric surgery carries a substantial evidence base for durable metabolic outcomes in complex cases — a position reinforced by documented GLP-1 treatment limitations in severe obesity.

05 — MELBOURNE MARKET CHARACTERISTICS

Structural Advantages for Metabolic & Bariatric Care

Private healthcare culture

Telehealth adoption

GP referrer density

High rates of private health insurance and self-funded specialist care. Strong disposition toward specialist-led pathways.	Among Australia's highest telehealth utilisation rates, supporting integrated metabolic care and GLP-1 monitoring models.	A concentrated GP ecosystem across inner and middle Melbourne — well-positioned to generate consistent specialist referral volume.
Clinical infrastructure Major private hospital infrastructure across the metropolitan area supports high procedural throughput and post-operative care continuity.	Health literacy Above-average health literacy and GLP-1 awareness in the Melbourne population relative to national benchmarks.	Wellness engagement High engagement with evidence-based health, nutrition and body composition — conducive to multidisciplinary metabolic models.

06 — MERIDIAN POSITIONING

Clinical Context & Practice Position

Meridian operates at the intersection of upper gastrointestinal surgery, bariatric care, and metabolic medicine. As GLP-1 medications broaden patient entry into weight management pathways, the clinical need for specialist surgical and metabolic assessment has not diminished — it has expanded. **Bariatric surgery carries a substantial evidence base for durable outcomes in severe obesity**, and combination care models are increasingly the clinical standard for complex metabolic cases. Meridian is structured to address this cohort: patients who have engaged with medical treatments, require specialist assessment, and benefit from continuity of care across surgical, metabolic, and multidisciplinary pathways.

MERIDIAN — GI & METABOLIC HEALTH, FITZROY · MELBOURNE